

VNSG 1500: CONCEPT BASED NURSING CARE I

CHANGES THROUGH THE LIFE SPAN: COMPLETED WORKSHEET

FLOW OF THE MAP: Injury Prevention feeds DOWN into Cognitive Development. Cognitive Development feeds UP into Physical Development and Health Promotion. Cognitive Development feeds DOWN-LEFT into Psychosocial Development and DOWN-RIGHT into Age-Activities. Psychosocial Development feeds RIGHT into Age-Activities.

PEDIATRIC CHANGES THROUGH THE LIFE SPAN

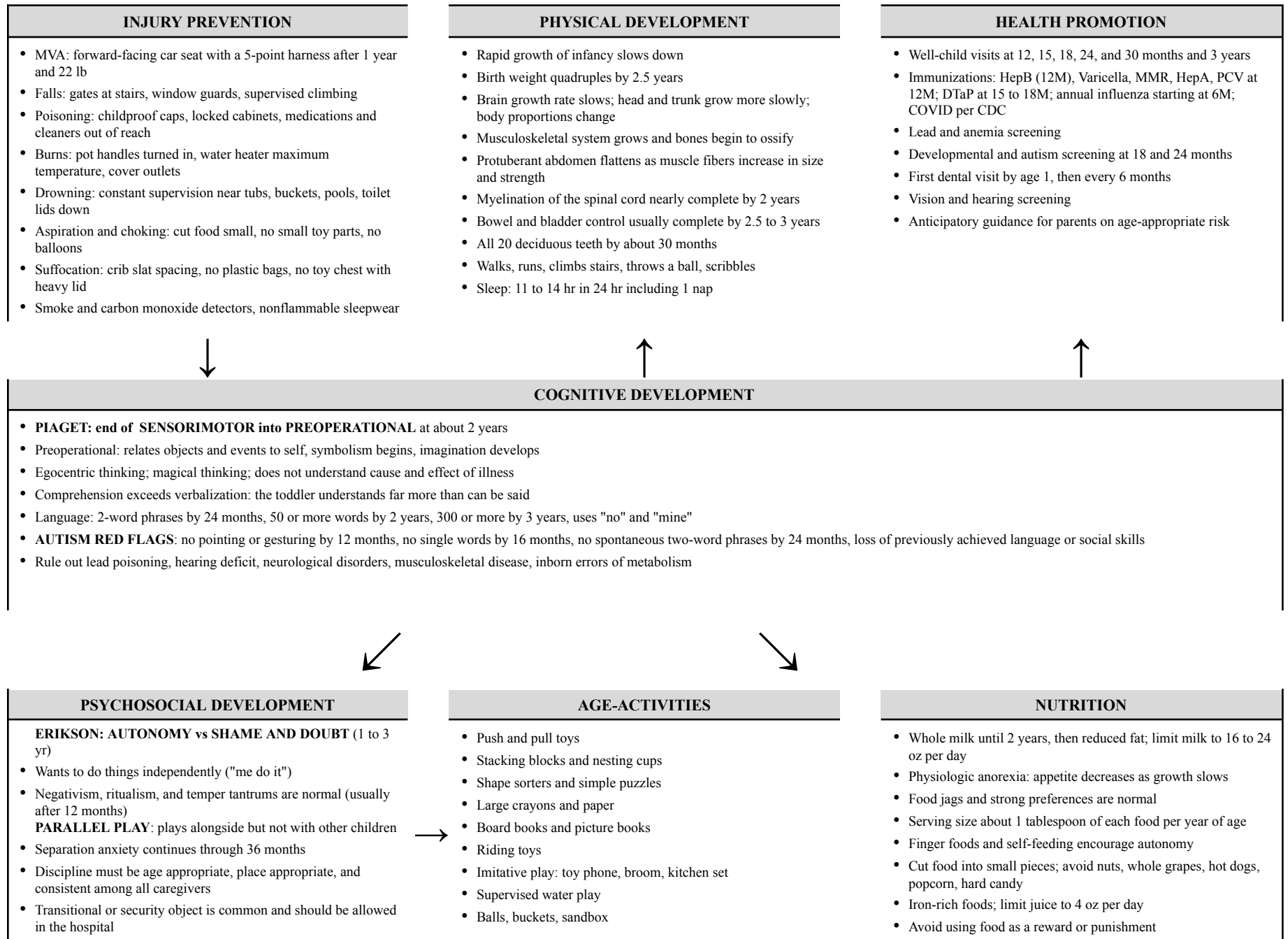
INFANT (NEONATES) 2D - 1 YEAR

INJURY PREVENTION	PHYSICAL DEVELOPMENT	HEALTH PROMOTION
• Aspiration of foreign objects: no small or removable toy parts, no nuts, grapes, hot dogs, popcorn • Bodily harm: never leave unattended on a flat surface, keep crib rails raised and locked • Burns: check formula and bath water temperature, set water heater maximum • Drowning: never leave unattended near water, protect from pools • MVA: rear-facing car seat until 1 year or 22 lb, never leave unattended in a car • Falls: protect from stairways, use gates • Poison: childproof caps, locked cabinets, poison control number posted • Suffocation: supine on a firm mattress, no pillows, bumpers, or loose bedding, no co-sleeping	• Fontanel: posterior closes 2 to 3 months, anterior closes 12 to 18 months • Infant size: head and chest circumference equal at 1 year • Weight: doubles by 6 months, triples by 1 year • Height: increases about 50 percent over the first year • Head circumference: measured at every well visit • Dentition: tooth eruption begins at 6 months, biting activities start • Sleep and rest: 15 hr day / 10 hr night, naps 1 to 2 per day; newborn sleeps in 4 hr intervals, up to 8 hr by 4 to 6 months • Motor: grasp reflex gone by 3 months, prehension 5 to 6 months, rolls over after 4 months, parachute reflex 7 to 9 months, pincer grasp by 1 year • Depth perception begins at 9 months; maternal iron stores decrease at 6 months	• Immunizations per CDC schedule • Check-ups: 48-hr, 1 wk, 2M, 4M, 6M, 9-12M (at least 5 physical exams in the first year) • Hearing and vision assessment as indicated • Required newborn screening tests • Growth grids and developmental screening • Nutritional counseling and parent education • Early Childhood Intervention (ECI) referral for delays, available to age 3
↓	↑	↑
COGNITIVE DEVELOPMENT		
PIAGET (SENSORIMOTOR) birth to 24M: reflexive, simple repetitive, imitation • Separation: infant begins to recognize self as apart from the caregiver • Object permanence: understands an object still exists when out of sight (about 8 to 9 months) • Mental representation: forms images of objects and actions late in the stage • Cause and effect develops through senses, body movement, and language LANGUAGE DEVELOPMENT: • crying, • vocalizes, • turns head to sound, • laughs, single vowel sounds, • two words, 3 to 5 words, • comprehension exceeds speech		
↙		↘

PSYCHOSOCIAL DEVELOPMENT	AGE-ACTIVITIES	NUTRITION
ERIKSON: TRUST vs MISTRUST (birth to 1 yr) • Success produces the virtue of hope; failure produces fear and insecurity • Consistency in meeting needs builds trust • A crying child should be soothed; infants cannot be spoiled • Freud oral stage: sucking brings comfort, hold during feedings, offer a pacifier if NPO or on IV nutrition • Separation anxiety normal from 6 to 36 months • Social smile, stranger anxiety around 8 months	• Rattles • Soft stuffed toy • Teething toy • Nesting toy • Pat-a-cake • Play with balls • Read books • Mirrors • Play with blocks • Sensory stimulation is essential to thought processes and perceptual ability	• Breastmilk • Formula • Solids at 6M, introduced one at a time every 4 to 7 days • Cereal (iron-fortified) first • Vegetables • Fruit • Finger foods • No cow milk, honey, or added salt or sugar before 1 year • Feed when hungry rather than delaying for a routine

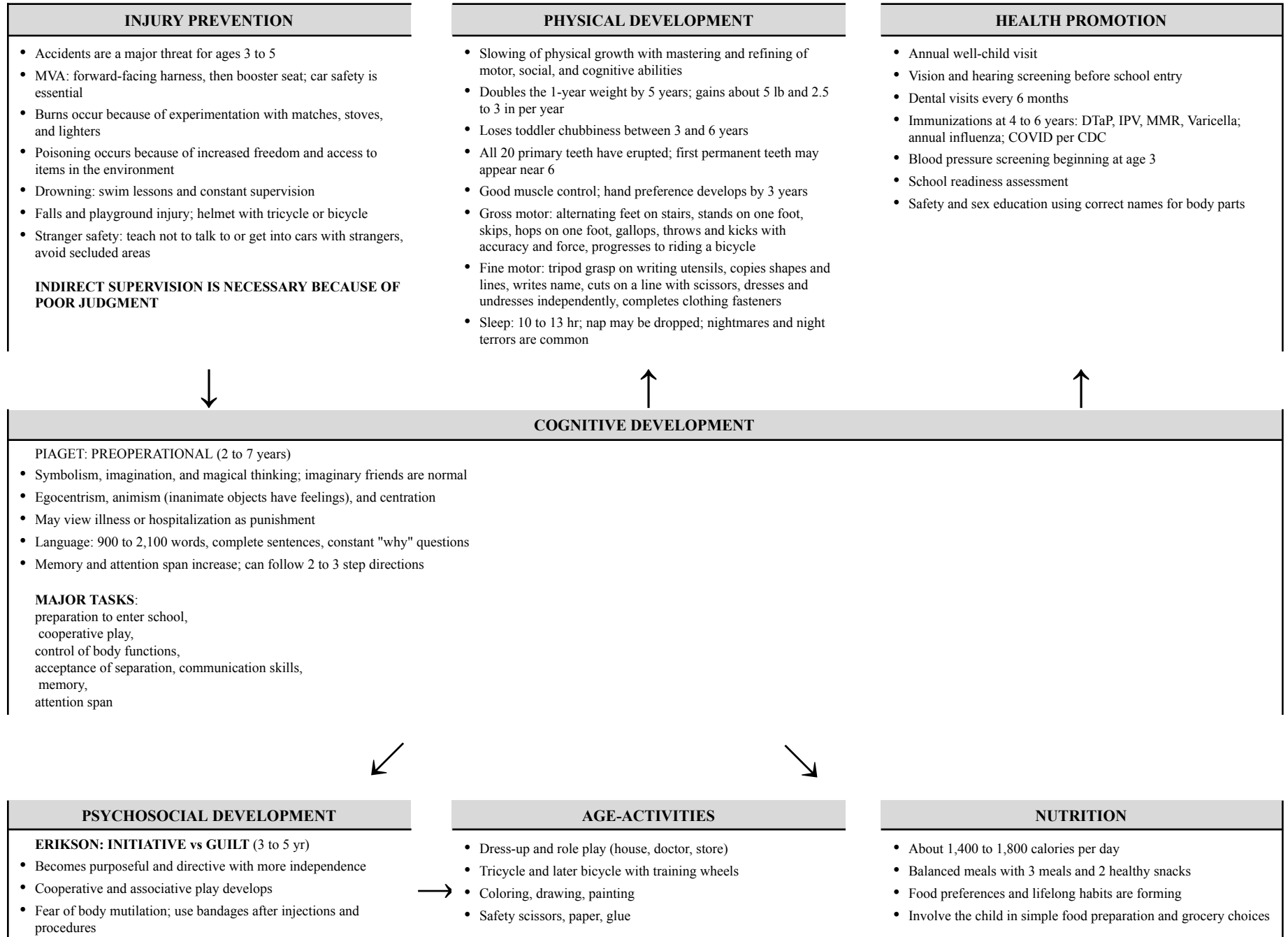
PEDIATRIC CHANGES THROUGH THE LIFE SPAN

TODDLERS 1-3 YEARS



PEDIATRIC CHANGES THROUGH THE LIFE SPAN

PRESCHOOLERS 3 - 6 YEARS



PLAY IS ESSENTIAL to physical, mental, emotional, and social development and should be NONCOMPETITIVE

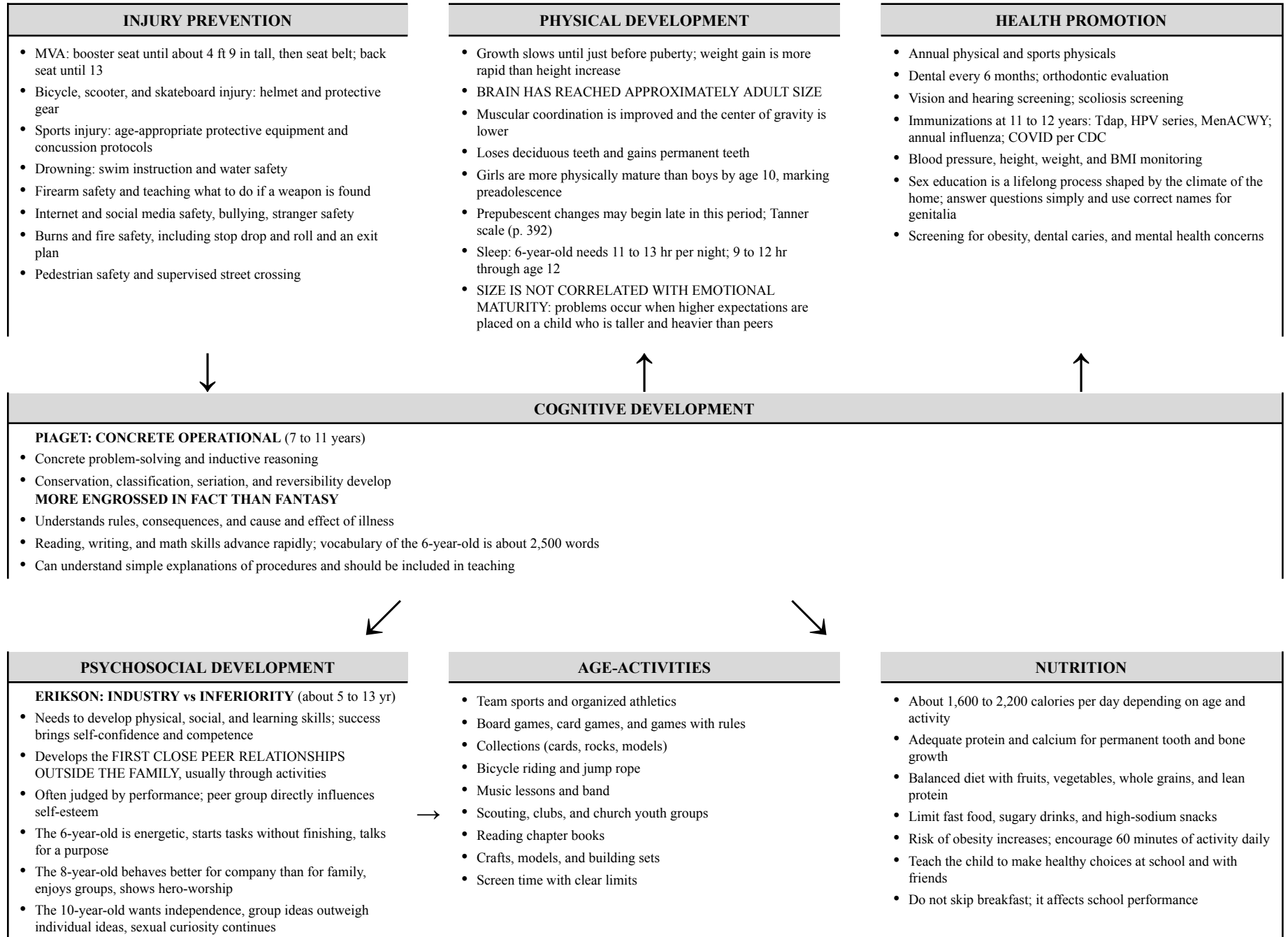
- Therapeutic play, play therapy, art therapy, music therapy; involve Child Life for procedures
- Consider health status, overstimulation and fatigue, and the diagnosis when choosing toys

- Puzzles and building blocks
- Simple board and card games
- Story books read aloud
- Sand and water play
- Medical play with dolls and teddy bears before procedures

- Limit sugary snacks, juice, and sodas; encourage water and milk
- Adequate calcium, vitamin D, and iron
- Family meals without screens; avoid pressuring the child to clean the plate

PEDIATRIC CHANGES THROUGH THE LIFE SPAN

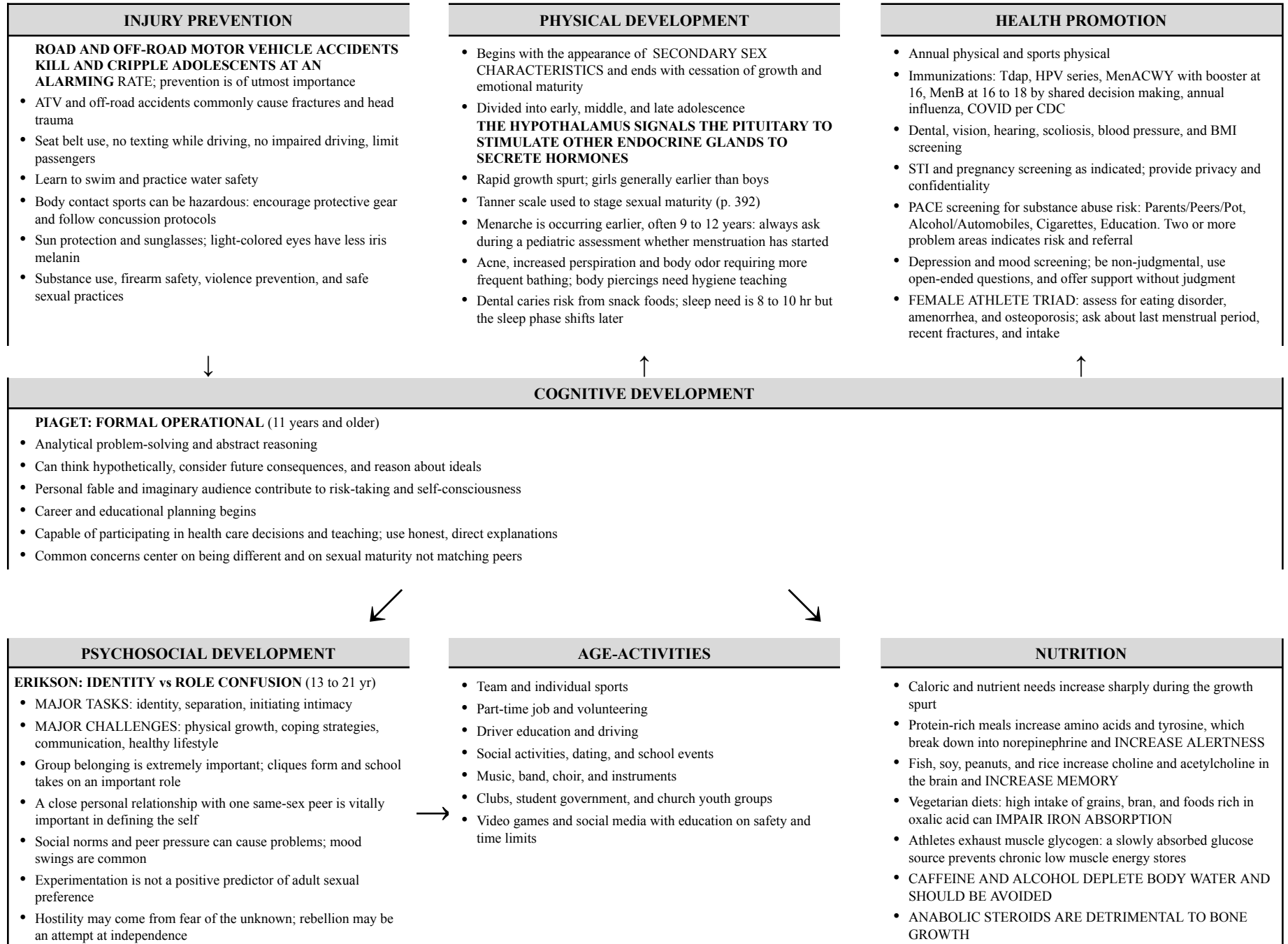
SCHOOL AGE 6 - 12 YEARS



- Sex role development is influenced by parents, society, and the school environment

PEDIATRIC CHANGES THROUGH THE LIFE SPAN

ADOLESCENTS 12 - 20 YEARS



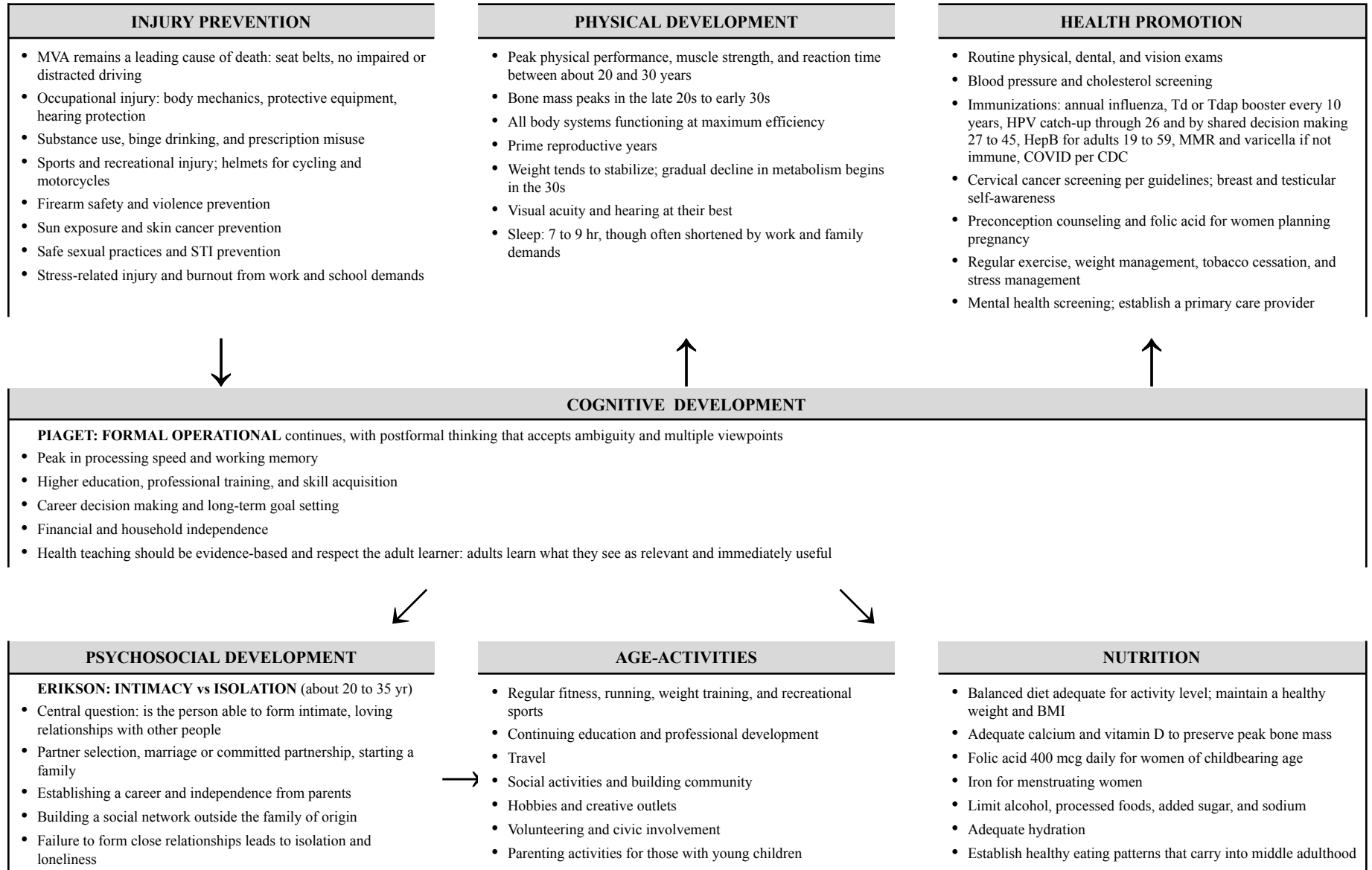
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• Increased calcium and iron needs; girls need iron after menarche
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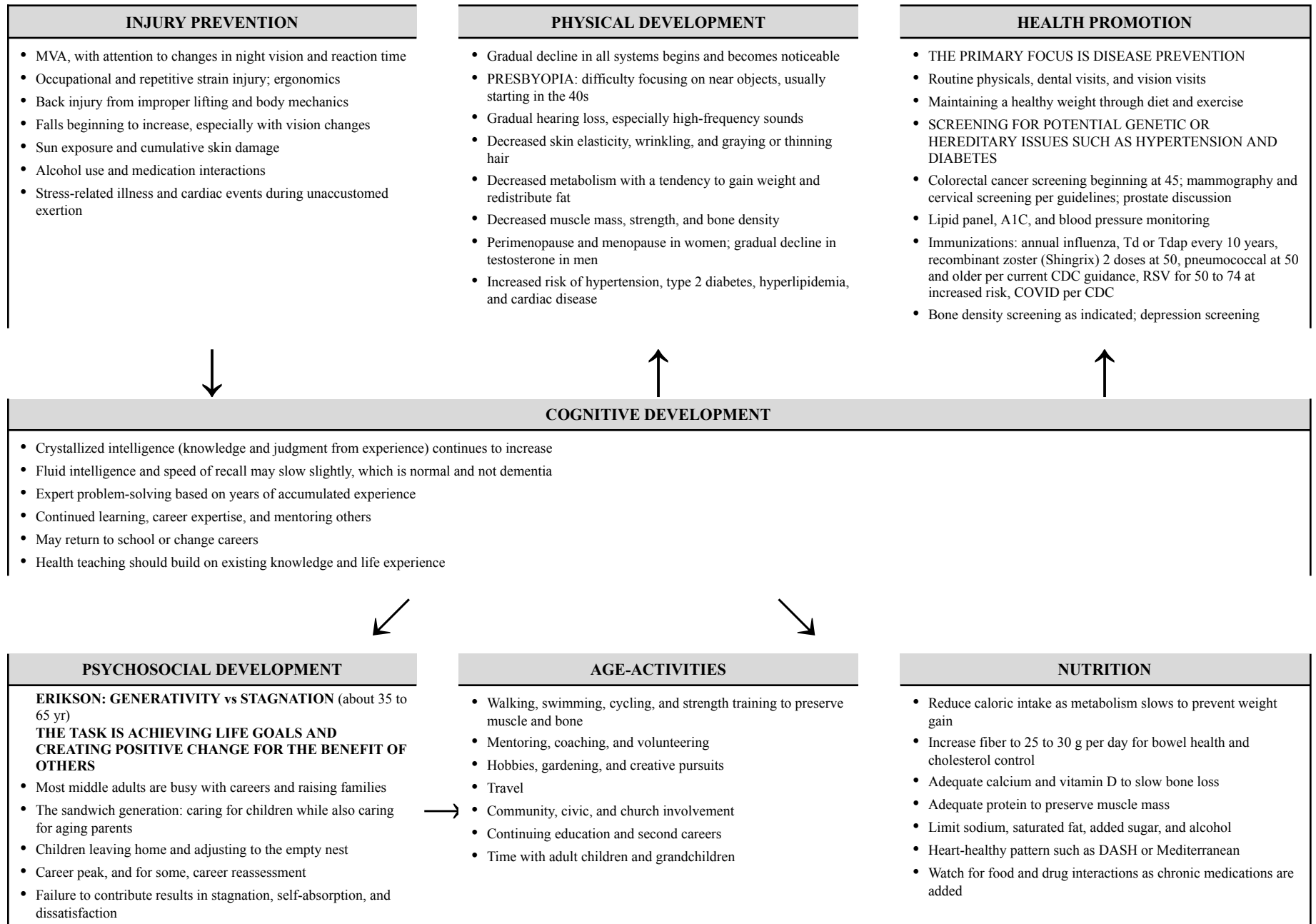
CHANGES THROUGH THE LIFE SPAN

YOUNG ADULT 20 - 35 YEARS

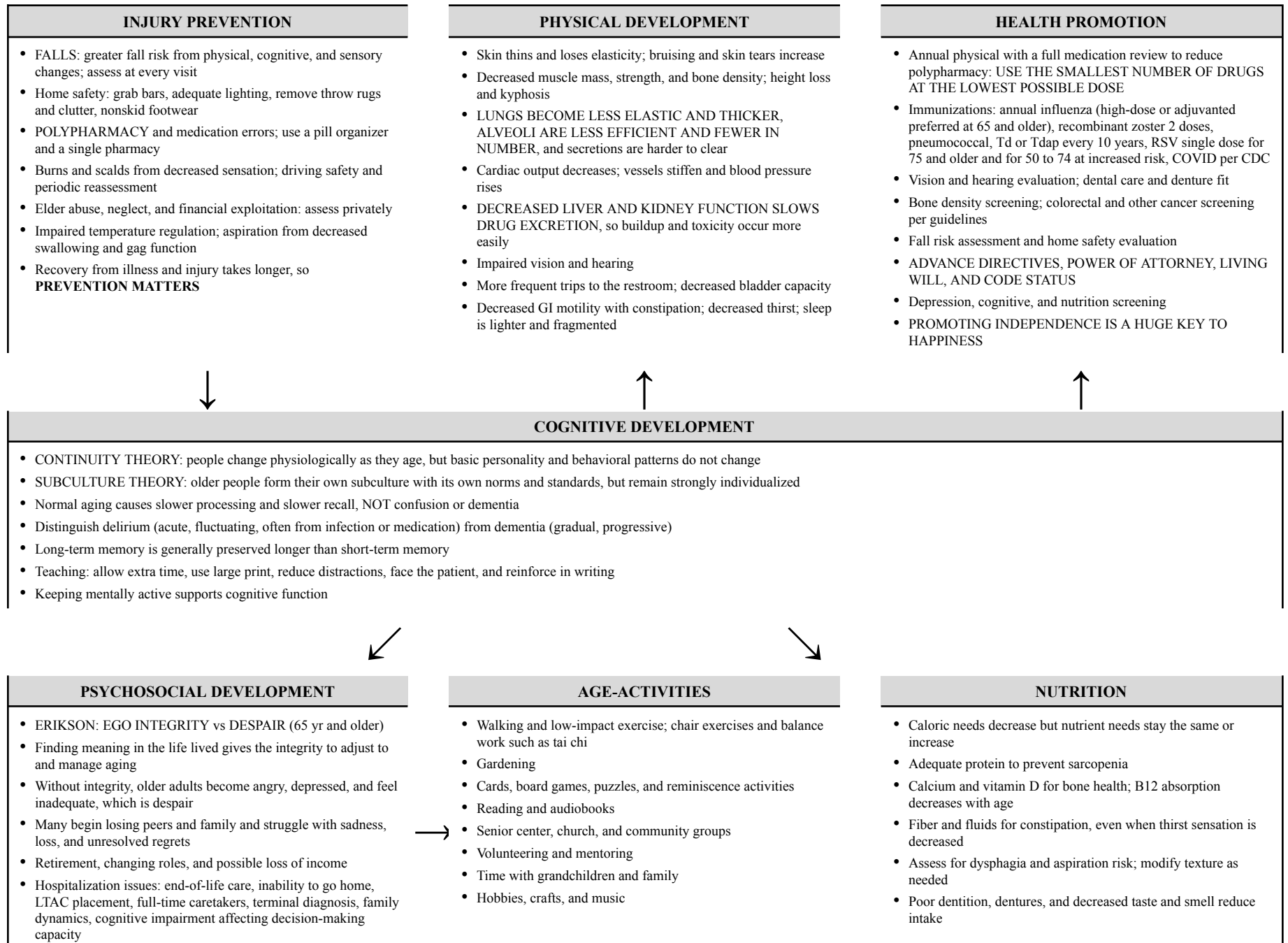


CHANGES THROUGH THE LIFE SPAN

MIDDLE-AGE ADULT 35 - 55 YEARS



CHANGES THROUGH THE LIFE SPAN OLDER ADULT (GERIATRIC) 55Y - DEATH



- Listen for what the patient is and is not saying; provide dignity and empower the patient and family

- Potassium replacement if taking a diuretic such as furosemide
- Watch food and drug interactions; limit sodium; assess isolation, transport, and cost as barriers

IMMUNIZATIONS

IMMUNIZATIONS - CENTERS FOR DISEASE CONTROL AND PREVENTION RECOMMENDS FOR HEALTHY INFANTS < 12M OF AGE INCLUDE:

AGE	VACCINE				
BIRTH	HEPATITIS B (HEP B)				
2 M	HEPATITIS B (HEP B) / DtaP (DIPHTHERIA, TETANUS, PERTUSSIS)	ROTAVIRUS (RV)	POLIO (IPV)	HAEMOPHILUS INFLUENZAE TYPE B (HIB)	PNEUMOCOCCAL (PCV)
4 M	DtaP	ROTAVIRUS (RV)	POLIO (IPV)	HAEMOPHILUS (HIB)	PNEUMOCOCCAL (PCV)
6 M	DtaP	ROTAVIRUS (RV)	POLIO (IPV)		PNEUMOCOCCAL (PCV)
INFLUENZA / COVID CANNOT BE GIVEN UNTIL >6M AGE	INFLUENZA 1ST DOSE, 2ND DOSE 30 DAYS LATER				
COVID - SEE CDC GUIDELINES	COVID 1ST DOSE, 2ND DOSE 30 DAYS LATER				
12 M	HEPATITIS B (HEP B)	VARICELLA (LIVE)	MMR (LIVE)	HAEMOPHILUS (HIB)	PNEUMOCOCCAL (PCV)
18 M	DtaP	HEPATITIS A (HEP A) #1			
2 Y	ANNUAL INFLUENZA	HEPATITIS A (HEP A) #2			
4 Y	DtaP	VARICELLA (LIVE)	POLIO (IPV)	MMR (LIVE)	

Note: doses at 12M and beyond are included to show the completion of each series.

WHAT IS THE DRUG USED FOR?

WHAT IS DRUG?	ABBREV.	WHAT IS THE DRUG USED FOR?
COVID-19	COVID	Prevents illness caused by the SARS-CoV-2 virus and reduces severe disease, hospitalization, and death. mRNA or protein subunit, not a live vaccine.
Diphtheria, Tetanus, and Pertussis	DTaP	Prevents diphtheria (severe throat membrane and airway obstruction), tetanus (lockjaw from soil-borne toxin), and pertussis (whooping cough). DTaP is for children under 7; Tdap and Td are the adolescent and adult forms.
Haemophilus influenzae type b	HIB	Prevents Haemophilus influenzae type b, a leading cause of bacterial meningitis, epiglottitis, pneumonia, and sepsis in young children.
Hepatitis A	HEP A	Prevents hepatitis A, a liver infection spread by the fecal-oral route through contaminated food and water. Two-dose series.
Hepatitis B	HEP B	Prevents hepatitis B, a bloodborne liver infection that can lead to chronic hepatitis, cirrhosis, and liver cancer. First dose is given at birth.
Influenza	INF	Prevents seasonal influenza and its complications, including pneumonia. Given annually. Inactivated (IIV) by injection or live attenuated (LAIV) intranasal.
Measles, Mumps, Rubella	MMR	Prevents measles, mumps, and rubella. LIVE vaccine, so it is contraindicated in pregnancy and in significant immunosuppression.
Pneumococcal	PCV	Prevents pneumococcal disease caused by Streptococcus pneumoniae, including pneumonia, meningitis, bacteremia, and otitis media.
Polio	IPV	Prevents poliomyelitis, a viral illness that can cause paralysis. Inactivated, given by injection in the United States.
Rotavirus	RV	Prevents rotavirus gastroenteritis, the leading cause of severe diarrhea and dehydration in infants. LIVE oral vaccine. Series must be started before 15 weeks.
Varicella	VAR	Prevents varicella (chickenpox) and reduces the risk of later shingles. LIVE vaccine, contraindicated in pregnancy and immunosuppression.

IMMUNIZATIONS - CENTERS FOR DISEASE CONTROL AND PREVENTION RECOMMENDS FOR CHILDREN >12 Y TO 18 Y

VACCINE	ABBREV.	RECOMMENDATION
Tetanus, Diphtheria, Pertussis	Tdap	1 dose at 11 to 12 years, then a Td or Tdap booster every 10 years.
Human Papillomavirus	HPV	Routine 2-dose series at 11 to 12 years (may start at 9). If the series is started at age 15 or older, 3 doses are required.
Meningococcal ACWY	MenACWY	1 dose at 11 to 12 years with a booster at age 16.
Meningococcal B	MenB	2 or 3 doses at ages 16 to 18 based on shared clinical decision making, or by indication.
Influenza	INF	1 dose annually.
COVID-19	COVID	Per current CDC guidelines.
Catch-up doses	HepB, HepA, MMR, VAR, IPV	Any doses not completed in childhood should be given as catch-up vaccination at the earliest opportunity.

IMMUNIZATIONS - CENTERS FOR DISEASE CONTROL AND PREVENTION RECOMMENDS FOR ADULTS 19 Y - 50 Y

VACCINE	ABBREV.	RECOMMENDATION
Influenza	INF	1 dose annually for all adults.
Tetanus, Diphtheria, Pertussis	Tdap / Td	1 dose of Tdap if not previously received, then a Td or Tdap booster every 10 years. Tdap is given during every pregnancy, at 27 to 36 weeks.
Human Papillomavirus	HPV	Routine catch-up through age 26. Ages 27 to 45 by shared clinical decision making.
Hepatitis B	HEP B	Recommended for all adults ages 19 to 59, and for those 60 and older with risk factors.
Measles, Mumps, Rubella	MMR	1 or 2 doses if born in 1957 or later and lacking evidence of immunity. LIVE, so not in pregnancy.
Varicella	VAR	2 doses if born in 1980 or later and lacking evidence of immunity. LIVE, so not in pregnancy.
Hepatitis A	HEP A	2 or 3 doses by risk factor or travel indication.
Meningococcal	MenACWY / MenB	By indication, such as asplenia, complement deficiency, college dormitory residence, or travel.
Pneumococcal	PCV	By indication for adults 19 to 49 with chronic conditions such as diabetes, chronic heart, lung, liver, or kidney disease, or immunocompromise.
COVID-19	COVID	Per current CDC guidelines.

IMMUNIZATIONS - CENTERS FOR DISEASE CONTROL AND PREVENTION RECOMMENDS FOR ADULTS AGE 50 > & AGE 65 + >

VACCINE	ABBREV.	RECOMMENDATION
Influenza	INF	1 dose annually. High-dose or adjuvanted formulations are preferred for adults 65 and older.
Recombinant Zoster (shingles)	RZV / Shingrix	2 doses for all adults 50 and older, and for immunocompromised adults 19 and older. Preferred over the older live zoster vaccine.
Pneumococcal	PCV15 / PCV20 / PCV21 / PPSV23	1 dose for adults 50 and older who have never been vaccinated, per current CDC guidance. The age was previously 65. If PCV15 is used, follow with PPSV23.
Tetanus, Diphtheria, Pertussis	Td / Tdap	Booster every 10 years.
Respiratory Syncytial Virus	RSV	Single dose for all adults 75 and older, and for adults 50 to 74 at increased risk of severe RSV disease.
COVID-19	COVID	Per current CDC guidelines, with the greatest benefit in adults 65 and older and those who are immunocompromised.
Hepatitis B	HEP B	For adults 60 and older with risk factors, including diabetes.
Measles, Mumps, Rubella / Varicella	MMR / VAR	Only if lacking evidence of immunity. Both are LIVE and are contraindicated in significant immunosuppression.

The CDC continually updates the immunization schedule. Always verify against the current schedule at [cdc.gov](https://www.cdc.gov) before administering.